PROJECT PROPOSAL

<The 16th e-ICON World Contest>

Team Name	MonAndo	**App Name**	MonAndo
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□ Summary of Web App

MonAndo is an AI-powered mental wellness platform that democratizes mental healthcare for underprivileged youth by delivering smartphone-based Virtual Reality (VR) exposure therapy, real-time AI voice guidance, and expressive tools like art therapy.

Target User:

The primary target audience consists of underprivileged youth and teenagers in developing nations (such as Bangladesh,Pakistan) and high-stress environments (such as South Korea) who :

- Face financial barriers
- Have geographic barriers (rural areas with no access to mental health professionals)
- Deal with social stigma around mental health (especially in South Korea due to Confucian cultural values)
- Include students struggling with academic anxiety, young people entering the workforce, and adolescents vulnerable to peer pressure and substance use
- socially marginalized young people who have been pushed to the edges of their educational and social systems through bullying, discrimination, or systemic exclusion

The app is also designed to serve anyone who lacks access to traditional psychiatric care, whether due to poverty, location, or cultural silence around mental health.

Main Function:

MonAndo's core innovation is affordable **Virtual Reality Exposure Therapy (VRET)** by utilizing a standard smartphone's internal gyroscope. VR therapy is clinically recognized for helping reduce depression,anxiety, social fear, PTSD symptoms, and phobias by placing users inside safe simulated environments where they gradually face stressful situations. Research shows VR therapy can significantly reduce anxiety symptoms and achieve therapeutic results comparable to traditional cognitive behavioral therapy (CBT). However, current VR therapy systems are expensive, require advanced equipment, and are usually available only in hospitals or private clinics.MonAndo solves this accessibility gap by transforming ordinary smartphones into low-cost therapeutic tools. Users can access immersive VR scenarios such as:

- Job interview simulations
- Classroom presentations
- Exam hall pressure
- Public speaking practice
- Crowded public transport
- Social interaction environments

The platform works both with inexpensive cardboard VR headsets and normal smartphone mode, making it scalable for low-income communities.

During sessions, an AI voice companion provides real-time calming guidance, breathing support, motivational feedback, and localized interaction in Bangla,Korean and English. The app also includes art therapy,music therapy, meditation, ASMR, progress tracking, and anonymous emotional support communities.

Unlike traditional healthcare systems, MonAndo is designed to be emotionally safe and non-judgmental. The app does not diagnose mental illnesses, prescribe medication, or replace psychologists and psychiatrists. Instead, it acts as a supportive therapeutic assistant and early emotional support platform that encourages users to seek help safely and privately.

To ensure ethical responsibility, the MonAndo concept was personally discussed with psychologists and psychiatrists, who positively reviewed the platform's accessibility-focused VR therapy model, emotional safety approach, and culturally localized design.

Thought Behind the App Name:

The name MonAndo is a deliberate fusion of two cultures and two languages. *Mon* (মন) is a Bangla word meaning mind or heart, the seat of emotion, thought, and inner life. *Ando* (안도) is drawn from the Korean word for comfort, relief, and emotional reassurance. Together, MonAndo means relief for the mind: a safe space where people can work through stress, anxiety, fear, and emotional pain without shame or judgment. The name itself embodies the app's core belief that healing should be accessible to everyone, in every language, in every culture.

□ Problem Statement and Background

Problem Statement:

Mental health has quietly become one of the most urgent public health crises of the 21st century, striking directly at the heart of **UN Sustainable Development Goal 3: Good Health and Well-being**. According to the World Health Organization, over 1 billion people worldwide currently live with a mental health condition, yet the vast majority never receive any form of treatment. This "treatment gap" reaches as high as 85% in Low- and Middle-Income Countries (LMICs), representing hundreds of millions of people suffering in silence and placing SDG 3's vision of universal well-being dangerously out of reach.

The economic cost is devastating. Depression and anxiety alone drain an estimated US $1 trillion from the global economy each year in lost productivity. Yet mental health receives only a fraction of healthcare budgets, with per capita mental health spending as low as US $0.04 in low-income

countries compared to up to US $65 in high-income nations. This extreme disparity means that geography and income largely determine whether a person ever receives help, directly violating **SDG 3.8**, which calls for universal access to quality healthcare services for all, including the most financially vulnerable.

According to UNICEF, children experiencing poverty face compounding mental health crises: chronic stress rewires developing brains, elevating cortisol levels that impair the hippocampus and trigger long-term emotional dysregulation. The National Center for Homeless Education further reports that approximately 1.2 million public school students experience homelessness each year, exposing them to severe psychological burdens, including trauma, family disruption, and chronic food insecurity.

Despite being a high-income country with world-class infrastructure, South Korea has one of the highest suicide rates among OECD nations, which is a direct failure of **SDG 3.4**, which specifically targets the reduction of mortality from mental health conditions and the promotion of mental well-being. The cause is not a shortage of hospitals; it is a deeply rooted cultural silence. Influenced by Confucian values that prize self-discipline and family honor, a staggering 93% of South Koreans with mental health conditions never seek professional help, with a mere 7% ever reaching out for therapy or psychiatric support. Admitting psychological distress is widely seen as a sign of personal weakness or family shame, and employers routinely screen out job applicants with documented mental health histories.

This silence is deeply philosophical in origin. Confucianism is followed across much of East Asia, is grounded in honoring one's ancestry and preserving family dignity. It emphasizes individual will, spiritual strength, and self-discipline, effectively framing mental illness as something to be endured rather than treated. The result is a society where young people, students, and workers silently bear anxiety, depression, and burnout rather than risk the devastating social consequences of being labeled mentally ill.



Bangladesh and other developing nations face a different but equally severe set of barriers. Here, the crisis is compounded by an extreme scarcity of mental health professionals, widespread poverty, and geographic isolation. Rural communities have virtually no access to psychiatrists or psychologists. For a family living below the poverty line, a single therapy session at US $65–150 per hour represents weeks of income.

The Promise and the Problem with VR Therapy: Virtual Reality Exposure Therapy (VRET) has emerged as one of the most clinically promising breakthroughs in addressing the unmet targets of **SDG 3.4**. By placing users inside safe, controlled simulated environments.Research consistently shows VRET achieves therapeutic outcomes comparable to traditional Cognitive Behavioral Therapy (CBT), and historically produces meaningful results at roughly half the cost of in-person clinical sessions. In 2006, the UK's National Institute for Health and Clinical Excellence (NICE) formally recommended VRET for patients with mild to moderate depression. For PTSD, social anxiety, and specific phobias, the clinical evidence is particularly strong.Yet for the communities that need it most, this technology remains completely out of reach and the promise of **SDG 3.4** and **SDG 3.8** remains unfulfilled. Entry-level standalone VR headsets cost between $300–$450 USD which is equivalent of weeks of food, rent, and utilities for a low-income family. Clinical software licenses add steep subscription fees on top of hardware costs. When combined with standard therapist session fees, advanced VRET becomes a luxury available only to the already privileged. Surveys reveal that only 0.1%–13% of mental health providers globally have ever used VR in clinical practice, with over 51%

reporting no prior experience at all. The technology exists but the access infrastructure does not.

The Failure of Existing Apps:

Over 10,000 mental health apps currently exist in global app stores. Yet over 70% of people who need mental health services still cannot access adequate care. The gap persists because existing solutions fail the populations that need them most in three critical ways :

First, they are not free. While many apps offer free downloads, core therapeutic features are locked behind paywalls and subscription fees, which makes it a defining barrier for underserved users and directly undermines **SDG 3.8**'s goal of financial risk protection in healthcare access.

Second, they are not localized. The dominant apps are designed around Western cultural norms, English-language interfaces, and social contexts largely foreign to users in Bangladesh, rural South Korea, or other non-Western communities. Therapeutic scenarios, AI voice companions, and community examples all reflect cultures the target users have never lived in.

Third, only about 3–4% of available mental health apps are supported by clinical evidence. The vast majority of adolescents and underprivileged users including those at risk of substance abuse targeted by **SDG 3.5** are relying on unverified tools that have never been properly tested on populations like theirs.

Our Solution:

We are planning to build MonAndo as a direct response to the unmet demands of **SDG 3.4**, **SDG 3.5**, and **SDG 3.8**.By utilizing the gyroscope and accelerometer already built into any standard smartphone, MonAndo delivers clinically grounded VRET at zero cost, transforming an ordinary mobile phone into a therapeutic tool without requiring any additional hardware. This directly advances **SDG 3.8** by removing financial and geographic barriers to evidence-based mental healthcare for underprivileged communities.



Featuring daily streak tracking, AI motivational support, and emotional emergency tools actively supports **SDG 3.5**, offering structured intervention for substance abuse rooted in the same clinical framework that drives the VR therapy engine. Through culturally localized AI voice companions speaking in Bangla and Korean, VR scenarios set in Dhaka classrooms and local markets, and a private anonymous interface that bypasses social stigma, MonAndo empowers users to take meaningful steps toward mental well-being fulfilling the promise of **SDG 3.4** in communities where that promise has, for too long, gone unmet.

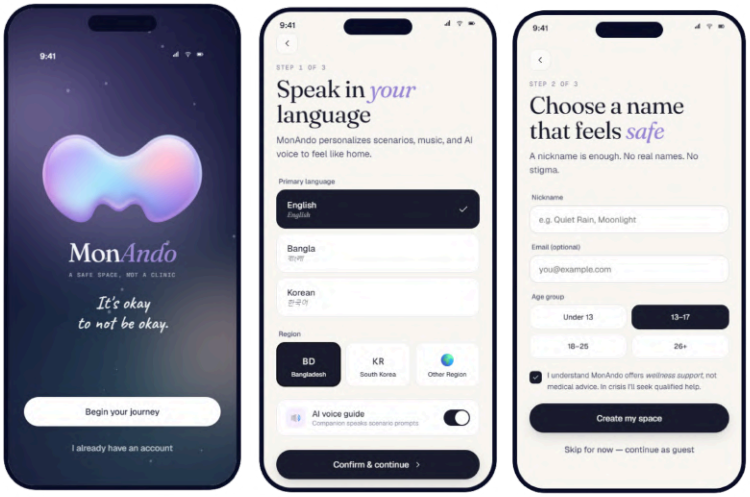
To ensure MonAndo is developed responsibly, the concept has been personally reviewed by local psychologists and psychiatrists; Prof. Dr. M S I Mullick of the Bangladesh psychiatric care and

Sharmin Akter of Bangladesh Mindsheba.They have positively assessed the platform's accessibility-focused VR therapy model, emotional safety approach, and culturally localized design. Their feedback has been incorporated into the app's core framework, and their guidance continues to shape its development. It is important to emphasize that MonAndo does not diagnose mental illnesses, prescribe medication, or act as a replacement for professional psychiatric care. Instead, it functions as a supportive therapeutic companion and early emotional support platform, a safe first step that encourages users to seek professional help when needed, on their own terms, without fear of judgment.



[Note: Image contains AI generated pictures]

□ Main Functions and UI / UX of the App

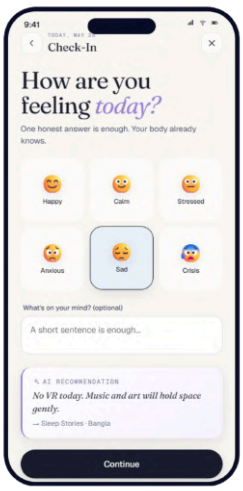


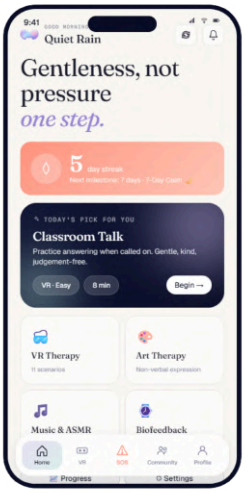
Screen: Splash/Welcome, Language-Region Setup and Login/SignUp

On the welcome screen, the user can view the app's core reassuring tagline and choose to either tap "Begin Your Journey" to start onboarding or select "I Already Have an Account" to log in.

After this screen, the user can select their preferred primary language and country from a simple localization menu.

On the next screen, the user can securely set up a profile by entering an anonymous nickname for identity and stigma protection, provide an optional email and password for backups, and select an age group for content filtering, or they can tap the permanently visible “Continue as Guest" button to preserve absolute privacy.

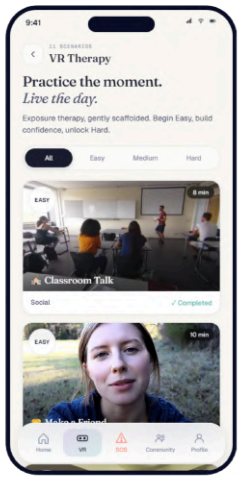




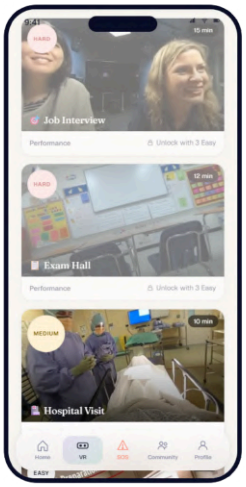
Screen: Mood Checking and Home Dashboard

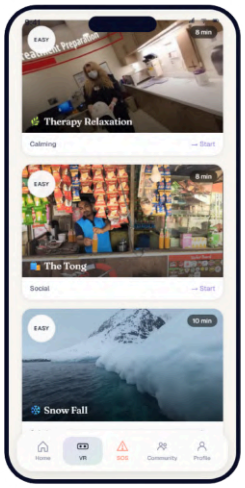
Every session begins here. A friendly AI avatar asks 'How are you feeling today?' The user's selection drives every subsequent recommendation. This screen also integrates optional smartwatch HRV data to make mood detection smarter.

On the home dashboard, the user can read a personalized greeting from the AI companion, check their active session streak, tap a prominent "Today's Pick" card to launch a recommended activity based on their mood, or navigate through a minimal 2×2 grid of module tiles (VR Therapy, Art Therapy, Music Hub, BioFeedback) using a persistent bottom navigation .





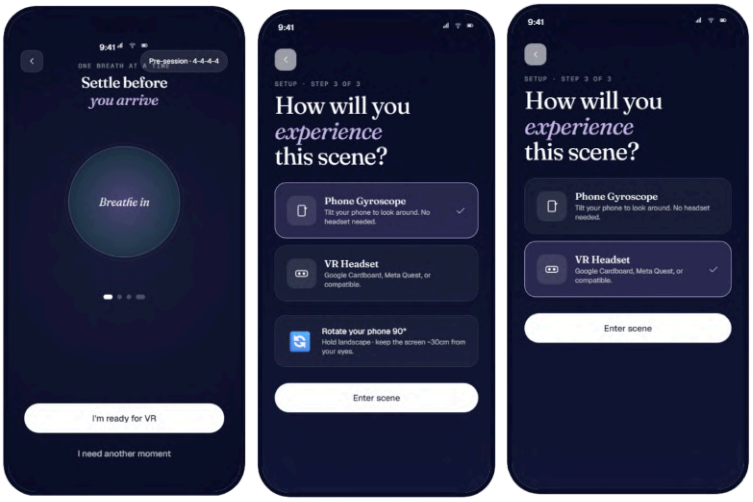


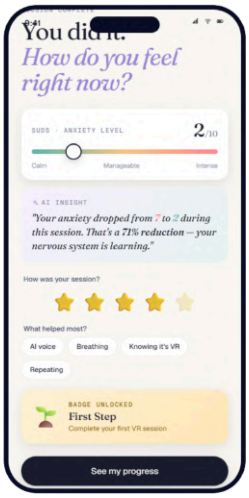
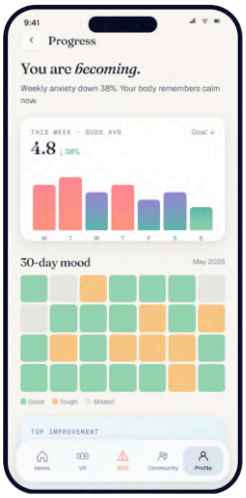
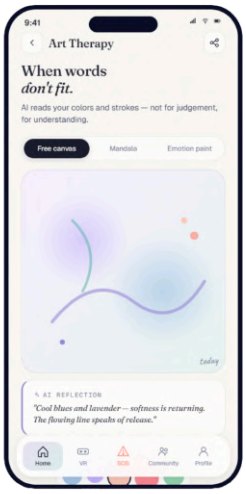
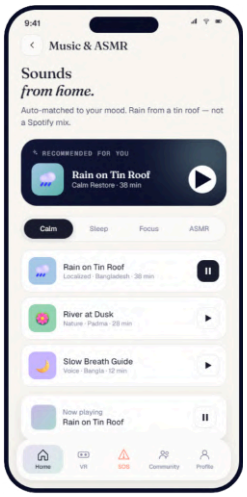


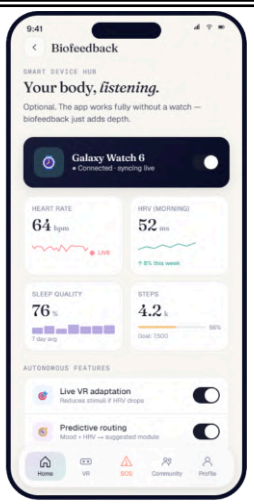
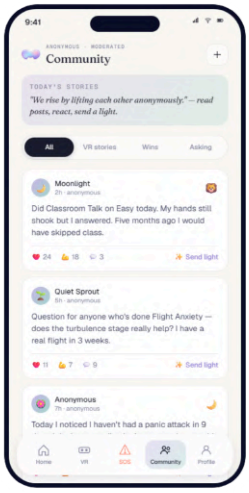
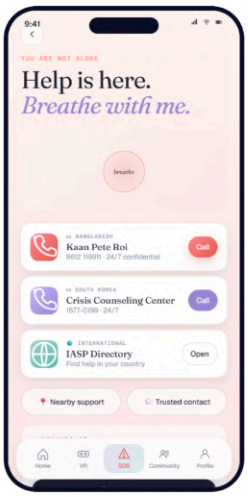
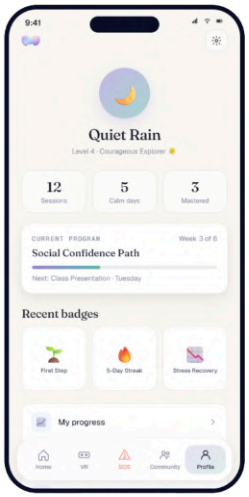
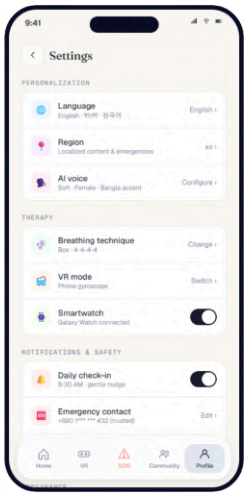
Screen: VR Scenario Selection Library

On this screen, the user can browse a scrollable catalog of VR scenarios organized by clinical difficulty (Easy, Medium, Hard), view their completion status, and select targeted exposure environments, such as:

- Classroom Talk
- Make a Friend
- Class Presentation
- Public Bus
- Job Interview
- Exam Hall
- Hospital Visit
- Therapy Relaxation
- Flight anxiety
- The Tong (Local Tea Stall)
- Snowfall

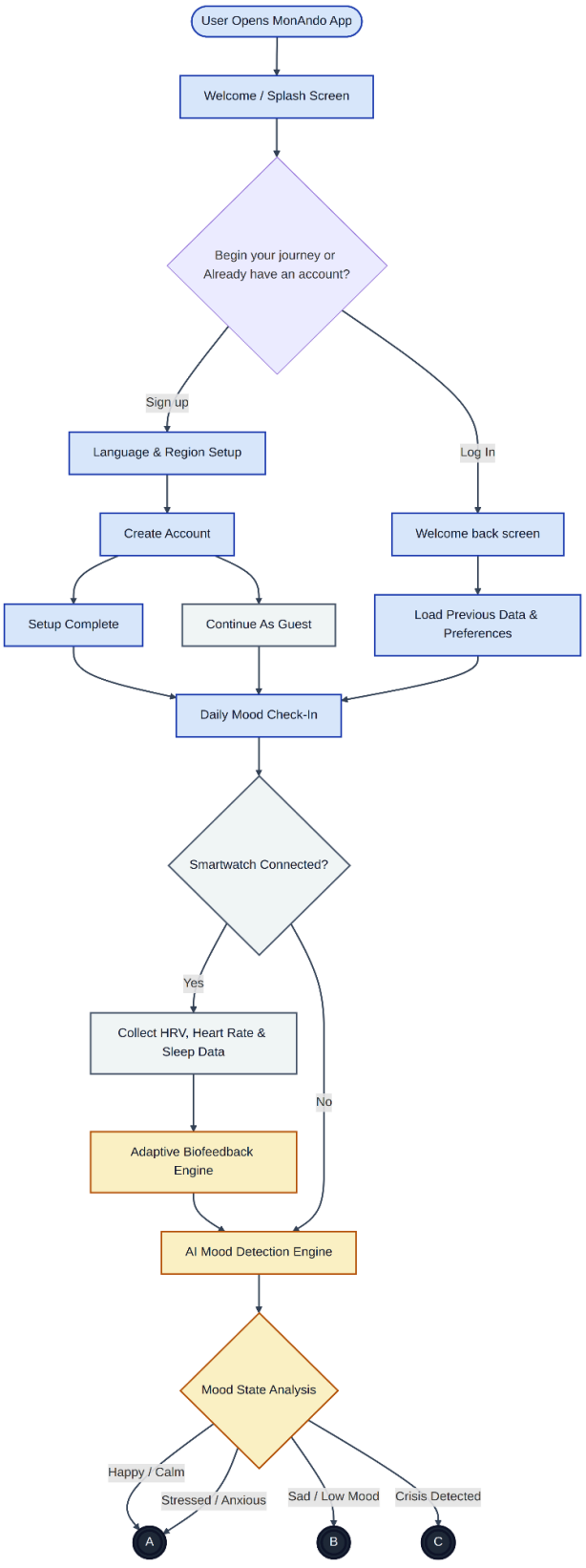
 Screen: Pre-Session Breathing Exercise and preferred hardware mode	On this screens, the user can follow an expanding visual ring to complete a mandatory 60-second box breathing (4-4-4-4) exercise to reduce baseline anxiety, select their preferred hardware mode (Phone Gyroscope Mode which prompts them to rotate their device 90° or VR Headset Mode), and tap "Enter VR Session" once they feel relaxed.
 Screen: Live VR Therapy Session (Smartphone Only)  Screen: Live VR Therapy Session (With VR Headset)	This is the core therapeutic experience. A 360-degree rendered VR environment with an AI voice companion guiding the user through the scenario in real time. Stress level is monitored (self-reported or via smartwatch). Sessions run 8–15 minutes.A large 'Pause / Exit' button is always visible. Tapping it immediately freezes the scene. The user can resume or exit to the debrief screen.

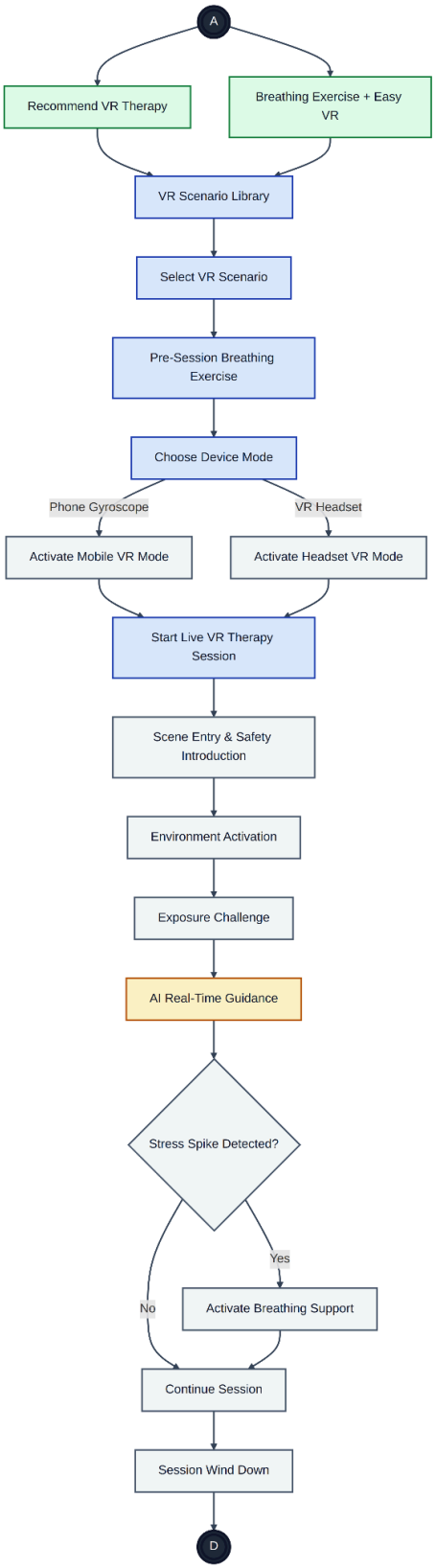
  Screen: Post-Session Debrief and Progress Tracker	On the Post-Session Debrief tab,user can log their final post-exposure anxiety level using an interactive 0–10 SUDS slider, type or voice-record a brief reflection about their experience, and read personalized AI-generated therapeutic insights that help build emotional self-awareness and feed directly into their progress dashboard. On the progress tab, the user can view a weekly anxiety bar chart that tracks downward trends, examine a color-coded 30-day mood history calendar, and review earned achievement badges, transforming complex clinical data into a warm, encouraging personal success story.
  Screen: Art and Music Therapy	In Art Therapy tab, the user can express non-verbal emotions by engaging in touch-based Free Drawing, Mandala Coloring, or Emotion Painting without needing any artistic skills. In the Music tab,the user can browse and stream a diverse library of mood-matched audio therapy divided into categorized channels like Sleep Hub, Focus Zone, Calm Restore, Meditation, ASMR Library, and Sleep Stories.

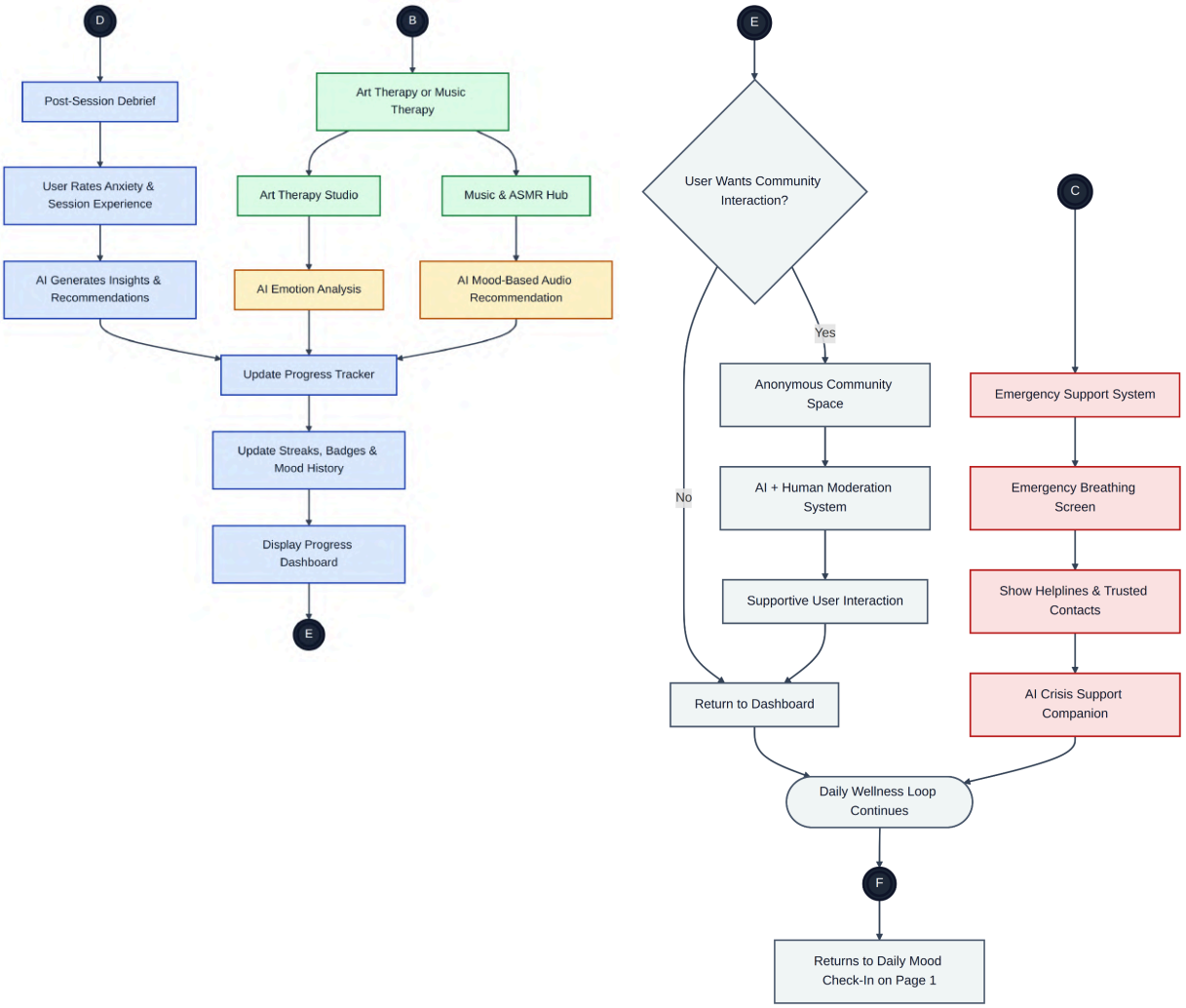
 Screen: Biofeedback	This integration is strictly optional; the application remains fully functional using only self-reported inputs.On this screen, the user can pair an external smartwatch via Bluetooth to sync objective heart rate, HRV, and sleep metrics, and toggle advanced systems like Dual-Data Objective Validation charts or Passive Panic emergency alerts.
  Screen: Anonymous Community Space and Emergency Support	On the anonymous community, the user can post short text updates about their mental health journey, browse peers' recovery breakthroughs without seeing photos or real names, and react using safe, predefined encouragement icons. On the emergency tab, which opens instantly whenever the user taps the permanently visible SOS button on any bottom navigation bar, the user can see regional helplines (like Kaan Pete Roi in Bangladesh and Suicide and Crisis Hotline in South Korea), and an AI chat companion to stay with them until they calm down.
  Screen: User Profile and Settings	On the profile tab, the user can manage their gamified wellness identity, track their active platform level, monitor total completed therapy sessions, view overall program progress percentages, and review their collection of unlocked reward badges. In Settings, the user can fully customize their app experience by configuring preferences for language, regional content, AI companion voice traits, box breathing pacing, default VR modes, notification permissions, emergency contacts, privacy filters.

□ User Flow of your Web App

Here is the flowchart showing the logic of the app:







Here is the step-by-step written workflow of our app:

Phase 1: Onboarding, Authentication & Mood Analysis

Step 1: Welcome & Setup: The user opens the MonAndo App and is greeted by the Welcome / Splash Screen. They are presented with two option: "Begin your journey" (Sign up) or "Already have an account?" (Log In).

Step 2: Authentication Path:

- **Sign Up:** New users proceed through Language & Region Setup and Create Account. From here, they can either complete the standard setup or choose to Continue As Guest.
- **Log In:** Returning users are directed to the Welcome back screen, where the app will Load Previous Data & Preferences.

Both paths seamlessly converge at the Daily Mood Check-In.

Step 3: Biofeedback & Mood Detection: The app checks if a device is paired or not

- **If Yes:** The app will Collect HRV, Heart Rate & Sleep Data, feeding it into the Adaptive Biofeedback Engine.
- **If No:** The workflow bypasses biofeedback. Data is then processed by the AI Mood Detection Engine, which performs a Mood State Analysis.

Step 4: AI Routing Branches: Based on the analysis, the user is routed to a specific logic branch:

- **Happy / Calm or Stressed / Anxious:** Routed to Branch A (VR Therapy).
- **Sad / Low Mood:** Routed to Branch B (Low-Friction Coping).
- **Crisis Detected:** Routed instantly to Branch C (Emergency Support).

Phase 2: The VR Exposure Therapy Flow (Branch A)

Step 5: VR Preparation: Upon entering Branch A, the app provides tailored initial guidance (Recommend VR Therapy or a Breathing Exercise + Easy VR). The user accesses the VR Scenario Library and must Select VR Scenario, followed by a Pre-Session Breathing Exercise.

Step 6: Device Mode & Session Entry: The user must Choose Device Mode:

- **Phone Gyroscope:** Activate Mobile VR Mode
- **VR Headset:** Activate Headset VR Mode

Once activated, the Start Live VR Therapy Session triggers, taking the user through Scene Entry & Safety Introduction and Environment Activation before facing the Exposure Challenge.

Step 7: Real-Time Guidance & Monitoring: During the challenge, the user receives AI Real-Time Guidance. The system actively monitors the user (via connected biofeedback or self-reporting) to determine: Stress Spike Detected?

- **If Yes:** The app will Activate Breathing Support before allowing the user to Continue Session.
- **If No:** The user will simply Continue Session.

The module concludes with a Session Wind Down, routing the user to Branch D.

Phase 3: Low-Friction Coping, Debrief & Dashboard

Step 8: Post-VR Debrief (Branch D): Users completing VR enter the Post-Session Debrief. The User Rates Anxiety & Session Experience, allowing the system to process the data so the AI Generates Insights & Recommendations.

Step 9: Art & Music Therapy (Branch B): Users routed here for low mood enter Art Therapy or Music Therapy:

- **Art Therapy Studio:** The user interacts with the canvas and art tools
- **Music & ASMR Hub:** The app provides a Mood-Based Audio Recommendation.

Step 10: Dashboard Updating: Both Branches B and D converge to update the user's profile. The app will Update Progress Tracker, followed by Update Streaks, Badges & Mood History. Finally, the

user lands on the Display Progress Dashboard. They are then routed to Branch E.

Phase 4: Community, Crisis Management & The Wellness Loop

Step 11: Community Interaction (Branch E):

- **If Yes:** They enter the Anonymous Community Space, protected by an AI + Human Moderation System, to engage in Supportive User Interaction. Afterward, they Return to Dashboard.
- **If No:** They bypass community features and directly Return to Dashboard.

Step 12: Emergency Support (Branch C) If a crisis was detected during Phase 1, the user bypasses all standard modules and enters the Emergency Support System. They are immediately shown an Emergency Breathing Screen, followed by a screen to Show Helplines & Trusted Contacts. Throughout this process, an AI Crisis Support Companion remains active.

Step 13: Closing the Loop Whether the user completes their dashboard/community interaction or successfully stabilizes through the crisis branch, all pathways eventually merge. The Daily Wellness Loop Continues, which ultimately Returns to Daily Mood Check-In on Page 1 for their next session.

□ Development Plan

All application screens will be designed in Figma, with supporting visual assets created in Adobe Illustrator. We will prototype the complete user flow within Figma before any code is written.

MonAndo will be built as a web-based application using React.js, ensuring full compatibility with modern mobile and desktop browsers without requiring any app store installation — any smartphone with a browser becomes a valid access point. For styling, we will use Tailwind CSS for fast, consistent, and responsive UI development.

Core infrastructure will be powered by Firebase. Firestore will handle real-time storage of mood logs, session history, and community posts. Firebase Authentication will manage secure logins and anonymous guest accounts. All media assets, including 360° panoramic images and audio files, will be hosted on Firebase Storage. Offline caching via service workers will support users with limited connectivity.

We will build immersive 360° exposure therapy environments using A-Frame, a WebXR framework that runs entirely within a standard mobile browser. Localized environments will be captured using an Insta360 X4 camera, processed in Insta360 Studio, and injected with spatial metadata before being streamed into our A-Frame scenes. Head tracking will be implemented by mapping the smartphone's built-in gyroscope and accelerometer directly to the A-Frame camera element, with optional cardboard VR headset support for enhanced immersion.

We will integrate the Claude API as a real-time therapeutic guide during VR sessions. The API will receive structured updates about the user's session state — active scenario, stress level, and session phase — and generate context-aware calming narration and grounding instructions. This output will be converted to speech via Google Text-to-Speech, with optional voice input captured by Google Speech-to-Text. The companion will support Bangla, Korean, and English.

The Art Therapy Studio will be built using Fabric.js, enabling touch-based drawing and mandala coloring. The Music and ASMR hub will stream mood-matched audio from Firebase Storage.

Optional smartwatch biofeedback is available via the Web Bluetooth API, with a spike detection algorithm triggering Emergency Support if panic-level readings are detected. All core features remain fully functional without a smartwatch.

Community posts will pass through the Claude API before publication, with self-harm content automatically flagged and hidden. We will use i18next for Bangla, Korean, and English UI strings, with the Translation API handling community post translation across languages.

In accordance with contest Rule 5: the Claude API is integrated as a functional component of the application, providing therapeutic guidance and content moderation. AI tools were also used as writing and research aids during proposal preparation. All core ideas, design decisions, and architecture were developed by the team.

MonAndo is committed to remaining completely free for all users. Initially, operational costs will be kept minimal by utilizing Firebase's free Spark tier, which covers early-stage database, authentication, and storage needs. Claude API usage costs will be managed through efficient prompt design, limiting API calls to essential moments.

For longer-term sustainability, we will apply for grants from organizations aligned with our mission, including UNICEF's Innovation Fund and national digital health programs in Bangladesh and South Korea. We will also seek partnerships with NGOs and schools that may sponsor platform access in exchange for anonymized, aggregated wellness insights. Socially responsible corporate sponsorships under CSR frameworks represent a further avenue we intend to explore.

□ Expected Impact of App on both your community and global society

Community Impact:

The community MonAndo is built for, underprivileged youth in Bangladesh and beyond, faces a mental health crisis that is largely invisible because silence has become the default. There are no local therapists to visit, no money to afford one if there were, and no cultural permission to admit the struggle in the first place. MonAndo enters this gap not as a substitute for professional healthcare, but as the first accessible bridge toward it.Here is how the platform provides immediate assistance to the local community:

- **Eliminating Financial Barriers:** The most immediate way the app helps is by removing the cost barrier entirely. A single therapy session in Bangladesh can cost anywhere from 65 to 150 US dollars, which for many families represents weeks of income. MonAndo delivers the same clinically validated technique, Virtual Reality Exposure Therapy, using the gyroscope already built into any standard smartphone. The hardware cost to the user is zero.
- **Overcoming Cultural Stigma and Ensuring Privacy:** Beyond cost, the app addresses the cultural silence problem. In many communities, visiting a psychologist carries a heavy social stigma. A documented mental health history can quietly close doors in employment and education. MonAndo's anonymous guest authentication and on-device encryption mean that a student practicing job interview confidence or working through exam anxiety leaves no trace. Their path to recovery cannot be discovered, shared, or used against them.
- **Academic and Professional Empowerment:** For students specifically, scenarios like the Classroom Talk, Class Presentation, and Exam Hall simulations target the precise moments of anxiety that cause school dropout and underperformance. A student who practices answering a

teacher's question fifty times in VR will walk into a real classroom with a fundamentally different nervous system response. For young people entering the workforce, the Job Interview simulation directly attacks one of the most economically devastating consequences of social anxiety: the inability to perform under high-stakes conditions that determine whether a person escapes the cycle of poverty.

- **Promoting Health-Seeking Behavior:** The app also addresses community health-seeking behavior through the Hospital Visit scenario. Fear of medical environments causes many people in rural Bangladesh to delay care until conditions become critical. Desensitizing users to clinical settings in VR, quietly and privately on their own phones, is expected to contribute to earlier visits and better health outcomes in communities where the app is deployed.
- **Resisting Peer Pressure and Substance Abuse :** The Tong scenario, set in a culturally familiar local tea stall environment, directly targets peer pressure around smoking and substance use among underprivileged youth. By repeatedly practicing refusal in a realistic, familiar scene with AI coaching in real time, users build the psychological muscle for the real moment.

Expected Changes Upon Successful Implementation:

If MonAndo reaches meaningful adoption across underprivileged youth communities, the expected changes operate at three levels: individual, community, and systemic.

At the individual level, students who engage consistently with the VR therapy scenarios are expected to show measurable reductions in social and performance anxiety. A student whose classroom anxiety drops from a self-reported 8 to a 4 over five sessions is not just feeling slightly better. They are participating more, learning more, and beginning to build the self-confidence.Students who use the Exam Hall and Job Interview scenarios are expected to perform better under pressure, reducing dropout rates linked to academic anxiety and increasing employment outcomes for youth who have practiced confidence in a consequence-free environment.

The addiction recovery module, through the Tong scenario and the daily streak tracker, is expected to produce measurable reductions in smoking initiation and substance use among vulnerable youth cohorts, directly advancing SDG Target 3.5. The anonymous community space is expected to reduce the acute isolation that worsens mental health for bullied, marginalized, and economically excluded adolescents, creating a micro-environment of peer support where none previously existed.

At the systemic level, MonAndo demonstrates a replicable model. If the platform is adopted through NGO partnerships, school programs, or government digital health initiatives, it has the potential to contribute meaningfully to closing the 85 percent treatment gap that defines the current global mental health crisis. The v2.1 Therapist Dashboard, planned for future development, will allow partnered psychologists and NGO counselors to review anonymized, aggregated session trends and identify users who may benefit from professional referral. This bridges the gap between digital self-help and clinical care in a scalable way.